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Correct Answer

Next Question

Practice Exam - 1

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A 20-year-old woman presents to clinic for counseling regarding her risk of developing hypertrophic cardiomyopathy (HCM). Her father was recently diagnosed with HCM at age 50 after presenting with aborted sudden cardiac death. She is concerned about her own risk and the potential risk to her future children and is willing to undergo genetic testing if indicated. Her screening transthoracic echocardiogram is normal. Her father undergoes genetic testing, which does not identify a pathogenic sarcomeric gene mutation associated with HCM.

- A. Her echocardiogram is normal and she is unlikely to develop the disease in her lifetime
-  B. She should undergo genetic testing
- C. She should ask her father to repeat genetic testing
-  D. She should undergo clinical evaluation with ECG and echocardiograms every 5 years starting now
- E. She should undergo clinical evaluation with ECG and echocardiograms every 5 years starting at age 40

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know how to guide genetic testing for HCM

Correct Answer: D: she should undergo serial clinical evaluation for HCM with ecg and echocardiograms intermittently at this time

Rationale: The patient reports a first degree family member with confirmed HCM. The family member is genotype negative. For patients with clinically confirmed HCM who have undergone genetic testing and were found to have no pathogenic variants, cascade genetic testing of their family members is not useful (Answer B). Similarly, if a genetic variant of unknown significance is identified, cascade genetic testing of a family is also not useful.

Answer A: The presence of a normal echocardiogram at this time does not preclude development of clinical HCM in the future.

Answer C: Repeating her father's genetic testing is unlikely to yield a different result not indicated at this time, as he was recently tested and no pathogenic variant was identified. Screening for HCM clinically when genetic testing is not available or is unlikely to be useful should occur at the time of diagnosis of HCM in a first degree family member.

Answer E: There is no indication to wait until age 40, and the patient would be exposed to risk by not entering a 3–5 year surveillance program immediately.

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